



Name : **MR.SHANKAR SHANA**
Age / Gender : 46 Years / Male
Ref.By : DR VIJAYA KUMAR P S
Req.No : BIL6018649

TID/SID : UMR3183378/ 29929293
Registered on : 14-Jul-2025 / 16:43 PM
Collected on : 14-Jul-2025 / 16:44 PM
Reported on : 14-Jul-2025 / 18:17 PM

TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Glycosylated Hemoglobin (HbA1C)

Investigation	Observed Value	Biological Reference Interval
Glycosylated Hemoglobin (HbA1c) Method:High-Performance Liquid Chromatography	11.2	Non-diabetic: <= 5.6 % Pre-diabetic: 5.7 - 6.4 % Diabetic: >= 6.5 %
Estimated Average Glucose (eAG) Method:Calculated	275	mg/dL

Interpretation: It is an index of long-term blood glucose concentrations and a measure of the risk for developing microvascular complications in patients with diabetes. Absolute risks of retinopathy and nephropathy are directly proportional to the mean HbA1c concentration. In persons without diabetes, HbA1c is directly related to risk of cardiovascular disease.

In known diabetic patients, HbA1c can be considered as a tool for monitoring the glycemic control.
Excellent Control - 6 to 7 %,
Fair to Good Control - 7 to 8 %,
Unsatisfactory Control - 8 to 10 %
and Poor Control - More than 10 %.

Reference: American Diabetes Association. Standards of Medical Care in Diabetes-2018.

* Sample processed at Tenet Diagnostics Regional Reference Lab, Municipal No.6A,27th Cross,7th Block, Jayanagar, Bengaluru 560070

--- End Of Report ---



Dr.M.G.Satish
Consultant Pathologist
KMC NO : 49885



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TEST REPORT

DEPARTMENT OF CLINICAL CHEMISTRY I

Creatinine, Serum

Investigation	Observed Value	Biological Reference Interval
Creatinine.	0.82	0.7-1.3 mg/dL
Method:Spectrophotometry, Jaffe - IDMS Traceable		

Interpretation:

Creatinine is a nitrogenous waste product produced by muscles from creatine. Creatinine is majorly filtered from the blood by the kidneys and released into the urine, so serum creatinine levels are usually a good indicator of kidney function. Serum creatinine is more specific and more sensitive indicator of renal function as compared to BUN because it is produced from muscle at a constant rate and its level in blood is not affected by protein catabolism or other exogenous products. It is also not reabsorbed and very little is secreted by tubules making it a reliable marker. Serum creatinine levels are increased in pre renal, renal and post renal azotemia, active acromegaly and gigantism. Decreased serum creatinine levels are seen in pregnancy and increasing age.

Biological reference interval changed; Reference: Tietz Textbook of Clinical Chemistry & Molecular Diagnostics, Fifth Edition.

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